

RIVERGATE REGIONAL TRAUMA CENTER

Level I Trauma Center
2100 Medical Center Drive, Rivergate, CA 91766

Reyes, Yolanda

MRN: RRTC-0074318 DOB: 06/22/1991 (34 yrs) Sex: F

Adm: 10/18/2025 D/C: 12/22/2025

Coverage: Medi-Cal (BIC on file) / BlueShield of California — HMO

DISCHARGE SUMMARY — TRANSFER TO INPATIENT NEUROLOGIC REHABILITATION

Discharge Date: 12/22/2025**Discharging Service:** Surgical / Neurosurgical Critical Care (SICU)**Discharge To:** Alder Creek Neuro-Rehabilitation Institute — 4501 Recovery Parkway, Rivergate, CA 91766 (inpatient neurologic rehabilitation)**Attending of Record (SICU / ongoing care):** Andrew Steinbach, MD — Attending Intensivist**Trauma Surgery Attending of Record (admission):** Priya Ramaswamy, MD**Receiving Physiatrist:** Helena Vasquez-Ortiz, MD (PM&R, Alder Creek Neuro-Rehabilitation Institute)

REASON FOR ADMISSION

High-energy auto-versus-pedestrian collision on 10/18/2025 at approximately 02:05 a.m., in which Ms. Reyes was struck by a transit bus while standing at a curbside bus stop, resulting in catastrophic polytrauma. Sixty-five-day surgical intensive care course now stabilized for transfer to inpatient neurologic rehabilitation for continued cognitive, mobility, swallow, and neurogenic-bladder rehabilitation.

DISCHARGE DIAGNOSES

1. Severe traumatic brain injury with diffuse axonal injury, resolving post-traumatic amnesia; persistent cognitive deficits in memory, attention, and executive function.
2. Status post open reduction and internal fixation of the left pelvic ring (percutaneous iliosacral screw fixation of the sacroiliac joint diastasis with anterior symphyseal plating).
3. Status post open reduction and internal fixation, bilateral lower extremities — reamed intramedullary interlocking nail of the right mid-shaft tibia; medial locking plate of the left distal tibial pilon with independent lag-screw fixation of the articular fragments; one-third tubular plate of the left distal fibula.
4. Status post left ankle soft-tissue reconstruction with pedicled medial gastrocnemius rotational flap and overlying split-thickness skin graft (Plastic Surgery, 10/25/2025).
5. Status post total splenectomy for AAST Grade V splenic laceration (10/18/2025); post-splenectomy vaccinations administered 10/28/2025; lifelong asplenia precautions.
6. Status post primary two-layer repair of intraperitoneal bladder-dome laceration (10/18/2025); watertight on follow-up cystography 10/28/2025.
7. Status post tracheostomy (11/05/2025) with successful decannulation (11/28/2025); stoma healing by secondary intention.
8. Status post percutaneous endoscopic gastrostomy (11/05/2025); PEG in use as primary route of nutrition, hydration, and medication delivery pending dysphagia rehabilitation.
9. Multiple right rib fractures with flail segment, right hemopneumothorax (resolved, chest tube removed 10/24/2025), and bilateral pulmonary contusions — resolved.
10. Hemorrhagic shock with acute traumatic coagulopathy — resolved.
11. Ventilator-associated pneumonia (MSSA, 10/27/2025) — treated to resolution.
12. Catheter-associated urinary tract infection (pan-susceptible *E. coli*, 11/02/2025) — treated to resolution.
13. Neurogenic bladder, pending urodynamic work-up in rehabilitation; on intermittent straight catheterization every six hours.
14. Dysphagia with silent aspiration on thin liquids (modified-barium-swallow study 12/16/2025) — NPO by mouth at transfer.
15. ICU-acquired weakness and protein-calorie malnutrition — improving on PEG feeds at goal.

16. Autonomic dysregulation / paroxysmal sympathetic hyperactivity secondary to diffuse axonal injury — controlled on scheduled propranolol.

17. Post-traumatic stress disorder screening deferred — patient currently unable to meaningfully participate in structured PTSD assessment given persistent cognitive impairment; formal screening (PCL-5 or clinician-administered instrument) to be re-attempted in rehabilitation once cognitive recovery permits reliable self-report.

HOSPITAL COURSE — CONDENSED

The full sixty-five-day course is summarized in the accompanying Hospital Course Summary of the same date. In brief: emergent damage-control laparotomy with splenectomy and bladder repair, combined-case external skeletal stabilization, decompressive craniectomy, and ICP monitor placement on 10/18/2025; staged orthopedic washouts on 10/22/2025; definitive pelvic and bilateral tibial ORIF with plastic surgical soft-tissue coverage on 10/25/2025; tracheostomy and PEG on 11/05/2025; ventilator liberation by 11/22/2025; tracheostomy decannulation on 11/28/2025. Neurologically, Ms. Reyes was maintained in a medically induced coma through approximately 11/07/2025, followed by a prolonged, agitated period of post-traumatic amnesia through the end of November, with slow one-directional cognitive improvement into December (GOAT 71 on 12/20/2025, still short of the emergence-from-amnesia cutoff of 75).

DISCHARGE MEDICATIONS

All medications delivered via PEG unless otherwise indicated.

Medication	Dose / Route	Indication
Levetiracetam	750 mg PEG BID	Post-traumatic seizure prophylaxis
Propranolol	20 mg PEG TID	Autonomic dysregulation / paroxysmal sympathetic hyperactivity
Quetiapine	25 mg PEG QHS	Agitation, sleep-wake regulation
Enoxaparin	40 mg subcutaneous daily	Venous thromboembolism prophylaxis
Pantoprazole	40 mg PEG daily	Stress-ulcer / PEG-tube prophylaxis
Docusate sodium	100 mg PEG BID	Bowel regimen
Senna	8.6 mg PEG QHS	Bowel regimen
Polyethylene glycol 3350	17 g PEG PRN daily	Bowel regimen
Acetaminophen	650 mg PEG q6h scheduled	Baseline analgesia
Oxycodone	5 mg PEG q6h PRN	Breakthrough pain, procedural mobilization
Ferrous sulfate	325 mg PEG daily	Anemia of inflammation, iron supplementation
Multivitamin with minerals	1 tab PEG daily	Nutritional supplementation
Cholecalciferol (Vitamin D3)	2,000 IU PEG daily	Bone health during immobilization
Bacitracin ointment	Topical to healing tracheostomy stoma BID	Wound care
Isotonic enteral formula	60 mL/hr continuous via PEG	Nutrition — approximately 25 kcal/kg/day, 1.5 g/kg/day protein

Held / discontinued at transfer: Insulin (off since 11/18/2025 with normoglycemia on tube feeds); dexmedetomidine, propofol, fentanyl infusions (discontinued with sedation weaning); cefazolin, ceftriaxone (courses completed); scheduled hydrocortisone (tapered off 10/21/2025).

Post-splenectomy vaccinations administered 10/28/2025: PPSV23, PCV20, Hib conjugate, MenACWY, and MenB series initiated. Booster schedule per protocol; recommend continuity with rehab-facility pharmacy and primary care.

Allergies: No known drug allergies.

FUNCTIONAL STATUS AT DISCHARGE

Cognition and communication. Persistent cognitive impairment consistent with resolving post-traumatic amnesia in the setting of severe traumatic brain injury. Intermittently oriented to self; not reliably oriented to place, time, or situation. Follows simple one-step midline commands with prompting; does not consistently follow multi-step commands. Attention span measured in seconds; short-term recall essentially absent; executive function not assessable at this time. Verbal output is limited by dysarthria and confusion, not by aphasia. GCS 13 (E4 V4 M5); GOAT 71 on 12/20/2025 (cutoff 75).

Decisional status. *Patient remains without decisional capacity at time of transfer.* Ms. Reyes has not at any point during this admission been assessed as decisional and did not participate in informed consent for any procedure or in any goals-of-care discussion. All consent has been obtained from her surrogate (see Family / Social, below). Serial neuropsychological evaluation to characterize the trajectory of cognitive recovery and to formally re-assess decisional capacity is deferred to the rehabilitation admission.

Activities of daily living. Dependent for all ADLs — feeding (PEG only, NPO by mouth), bathing, dressing, toileting, and grooming. Requires two-person maximum-assistance transfers with mechanical lift for bed-to-chair activity. Requires continuous 24-hour supervision for safety given cognitive impairment, aspiration risk, non-weight-bearing status, and impulsivity during periods of agitation.

Mobility. Non-ambulatory. Non-weight-bearing bilateral lower extremities; anticipated advancement to touch-down weight-bearing on the right at approximately eight weeks post-fixation and continued non-weight-bearing on the left pending twelve-plus-week clinical and radiographic milestones. Bilateral custom knee immobilizers in place for transfers.

Airway / pulmonary. Room air, saturating 97–99%; productive cough; healing tracheostomy stoma covered with a dry dressing; no supplemental oxygen requirement.

Swallow / nutrition. NPO by mouth secondary to silent aspiration on thin liquids per 12/16/2025 modified-barium-swallow study. All nutrition, hydration, and medications via PEG. Dysphagia rehabilitation to continue at receiving facility.

Bladder / bowel. Voiding by intermittent straight catheterization every six hours with post-void residuals 60–120 mL; neurogenic-bladder work-up pending. Bowel regimen effective; no incontinence issues in the acute phase.

Skin. Intact. Sacral pressure injury healed. Left ankle rotational flap and STSG well-perfused and healed. Surgical incisions clean, dry, and intact.

PTSD screening. As above, formal screening deferred; cognitive impairment precludes reliable self-report at this time. Trauma-informed care approach to be continued in rehabilitation.

REHABILITATION PROGNOSIS

Long-term inpatient neurologic rehabilitation will be required, with an anticipated initial inpatient rehabilitation stay measured in months rather than weeks given the combined burden of severe TBI, dysphagia, neurogenic bladder, and the extended non-weight-bearing orthopedic recovery. Cognitive trajectory has been slow but one-directional to date; the extent of ultimate cognitive, functional, and vocational recovery cannot be reliably predicted at this stage and remains uncertain. Full return to baseline cognition, independent ambulation, and independent decision-making is not assured and may not be achievable. Serial neuropsychological evaluation, physiatric re-assessment, and interdisciplinary goal-setting at the receiving facility will refine prognosis over the coming weeks and months. Anticipatory guidance to the family has been provided consistent with this uncertainty.

FAMILY / SOCIAL

Ms. Reyes lives alone at her pre-injury residence in Rivergate. Prior to injury she was employed full-time as a sales associate (retail).

Her closest family member and identified surrogate throughout this admission has been her sister, **Ms. Marisela Reyes**, who has been engaged with the trauma team, trauma social work, and case management from the day of admission and who has provided all substitute consent for operative and procedural care. A **temporary conservatorship of the person** was granted by the Los Angeles County Superior Court (probate) on **11/10/2025** with Ms. Marisela Reyes appointed as temporary conservator; that appointment ratified the earlier surrogate consents on file and has served as the source of consent for all subsequent care through the date of transfer. The temporary

conservatorship remains in place at the time of discharge and has been communicated in writing to the receiving facility's admitting team and social work department. Copies of the probate order have been included in the transferred medical record.

Ms. Marisela Reyes has been the point of contact for all care conferences and family meetings and will continue in that role at the receiving facility.

STATUS AT TRANSFER (12/22/2025)

- **Neurologic:** Persistent severe cognitive impairment; GCS 13; GOAT 71 (12/20/2025); intermittently oriented to self only; requires continuous 24-hour supervision; *without decisional capacity*.
- **Airway / Pulmonary:** Room air; tracheostomy decannulated 11/28/2025; stoma closing by secondary intention.
- **Cardiovascular:** Normotensive; sinus rhythm 78–92 bpm on propranolol.
- **GI / Nutrition:** PEG-fed at goal; NPO by mouth pending swallow rehabilitation.
- **GU:** Intermittent straight catheterization every six hours; neurogenic-bladder work-up pending.
- **Musculoskeletal:** Non-weight-bearing bilateral lower extremities; healing surgical incisions; left ankle flap / STSG intact.
- **Skin:** Intact; sacral pressure injury healed.
- **Functional:** Dependent for all ADLs; two-person maximum-assist transfers with mechanical lift; non-ambulatory.
- **Infectious:** No active infection; all cultures negative at transfer.
- **Psychiatric / PTSD:** Formal screening deferred; trauma-informed care to continue in rehabilitation.

Condition: Stable for transfer to inpatient neurologic rehabilitation.

Code status at transfer: Full code, per surrogate.

FOLLOW-UP

- **Physical Medicine & Rehabilitation:** Care transferred to Dr. Helena Vasquez-Ortiz, MD, at Alder Creek Neuro-Rehabilitation Institute.
- **Orthopedic Trauma Surgery (Dr. Sundararajan):** outpatient clinic at four weeks post-transfer; interval imaging as scheduled by rehab team.
- **Plastic and Reconstructive Surgery (Dr. Hasegawa):** six-week follow-up for left ankle flap / STSG.
- **Otolaryngology – Head & Neck Surgery (Dr. Vaccaro):** as-needed follow-up for stoma healing; sooner if signs of stenosis or delayed closure.
- **General Surgery (Dr. Kirkham):** PEG follow-up; assess for downsizing / removal once oral intake resumes.
- **Urology (Dr. Nordstrom):** outpatient urodynamics after neurogenic-bladder stabilization.
- **Neurosurgery / Neurology:** signed off 12/18/2025; outpatient neurology follow-up in three months, or sooner for seizure activity.
- **Primary care:** to be established through the receiving facility for post-splenectomy vaccine booster tracking and long-term coordination.

Warm hand-off performed by telephone at 13:35 on 12/22/2025 to Dr. Helena Vasquez-Ortiz, MD, receiving physiatrist. PEG regimen, wound care, medication reconciliation, orthopedic weight-bearing precautions, seizure precautions, aspiration precautions, and surrogate-consent framework communicated in writing and confirmed verbally. Copies of the operative report compilation, current imaging, the probate conservatorship order dated 11/10/2025, and the SICU medication administration record accompany the patient.

Electronically signed by:

Andrew Steinbach, MD

Attending Intensivist — Surgical / Neurosurgical Critical Care

Rivergate Regional Trauma Center

12/22/2025 14:35